

****DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 06

Age: 49 years old

Sex: Female

MRN: 000-TEST-06

Admission Date: [DATE]

Discharge Date: [DATE]

Length of Stay: 3 days

****PRIMARY AND SECONDARY DIAGNOSES****

1. Congestive Heart Failure with reduced ejection fraction (EF 32%)
2. Hypothyroidism — stable on current regimen
3. Generalized Anxiety Disorder — managed pharmacologically

****HOSPITAL COURSE****

Patient 06 was admitted via ED with chief complaint of progressive dyspnea on exertion (DOE) × 10 days and lower extremity edema. Initial vital signs: BP 158/92, HR 88, RR 20, SpO2 92% on RA. Physical exam notable for 2+ bilateral ankle edema, JVD, and scattered bibasilar crackles.

Diagnostic workup performed:

— Transthoracic echocardiogram: EF 32%, moderately dilated LV, Grade II diastolic dysfunction

- BNP: 487 pg/mL (markedly elevated)
- CBC, CMP: unremarkable; K⁺ 4.3, Cr 0.9
- Chest X-ray: mild pulmonary edema, no acute consolidation

Therapeutic interventions initiated:

- IV furosemide 40 mg daily × 2 days; transitioned to oral diuretic therapy
- Initiated ACEI (lisinopril) for afterload reduction and neurohormonal modulation
- Beta-blocker (carvedilol) uptitrated to target dose
- Counseled on fluid restriction (2L/day), daily weights, and dietary sodium limitation (<2g/day)
- Diuresis achieved; patient ambulating without difficulty by day 3
- Discharged in stable condition with improved dyspnea and resolution of orthopnea

****DISCHARGE MEDICATIONS****

Medication	Dose	Route	Frequency	Duration
Furosemide	40 mg	PO	BID	Ongoing
Lisinopril	10 mg	PO	q.d.	Ongoing
Carvedilol	12.5 mg	PO	BID	Ongoing
Levothyroxine	75 mcg	PO	q.d.	Ongoing
Sertraline	50 mg	PO	q.d.	Ongoing
Potassium chloride	20 mEq	PO	q.d.	Ongoing

****ACTIVITY & WEIGHT-BEARING STATUS****

- No strenuous exercise; walking as tolerated

- Avoid heavy lifting (>10 lbs) × 4 weeks
- Gradual return to ADLs over next 2 weeks

****DIETARY RESTRICTIONS****

- Strict fluid restriction: 2 liters daily
- Sodium-restricted diet: <2 grams per day (avoid processed foods, canned goods, added salt)
- Encourage heart-healthy diet rich in lean proteins and vegetables

****MONITORING INSTRUCTIONS****

- Daily morning weight; report gain >2–3 lbs in 24 hours or >5 lbs in 1 week
- Monitor BP and HR at home; record values
- Strict medication adherence critical for CHF management

****RED FLAG SYMPTOMS — SEEK EMERGENCY CARE IF:****

- Acute SOB at rest or with minimal activity
- Chest pain or pressure
- Syncope or near-syncope
- Irregular heartbeat with palpitations or presyncope
- Severe orthopnea or inability to